Credit Valley: ED performance

Latest month: July 2026 · Source: Ontario Health (NACRS, CIHI)

Location: Credit Valley | Mississauga | Queensway (UCC)

Monthly ED visits

9,961

Time to first doctor (h)

2.7

Ontario 2.0

Admitted: time in ED (h)

26.0

Ontario 17.7

Admitted: left ED within 8 h

15.4%

Ontario 28.2%

Scorecard vs Ontario and provincial targets

Measure	Site (h)	Ontario (h)	Gap (h)	% Within Target	Status	Ontario rank	vs last year (h)
Time to first doctor assessment	2.7	2.0	0.7		No target	#130 of 161	0.3
Length of stay: low urgency, sent home	3.9	3.2	0.7	60.9%	Missed	#126 of 164	0.5
Length of stay: high urgency, sent home	5.7	4.7	1.0	80.7%	Near	#138 of 163	0.3
Length of stay: admitted patients	26.0	17.7	8.3	15.4%	Missed	#147 of 154	1.0

Key takeaways

- **Admitted patients wait longest.** Across Ontario, only about 28% of admitted patients leave the ED within the 8-hour target. Credit Valley (15%) and Mississauga (27%) both miss it.
- **Credit Valley ranks in the bottom fifth province-wide on every measure**, even though its patient volume and mix are almost identical to Mississauga's. That points to how the site runs (bed flow, staffing) rather than who arrives.
- **Mississauga improved.** Its admitted patients' average time in the ED fell by 3.9 h over the past year.
- **Queensway is an urgent care centre**, which explains its fast results. It isn't directly comparable to a full ED.

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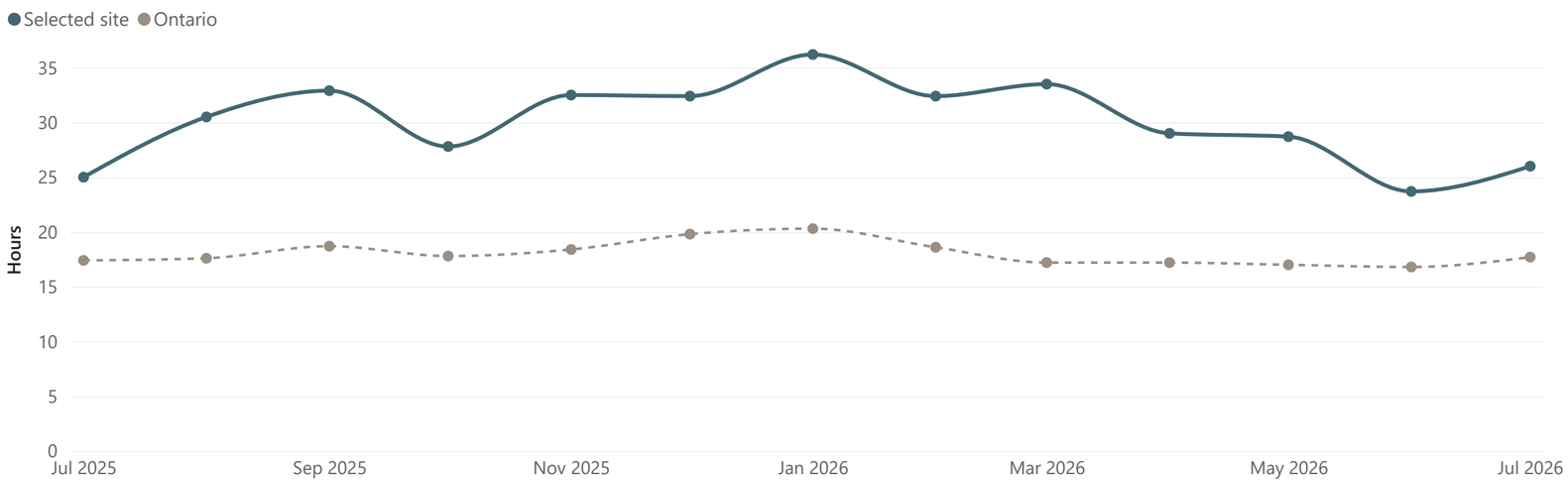
Measure

Length of stay: admitted patients

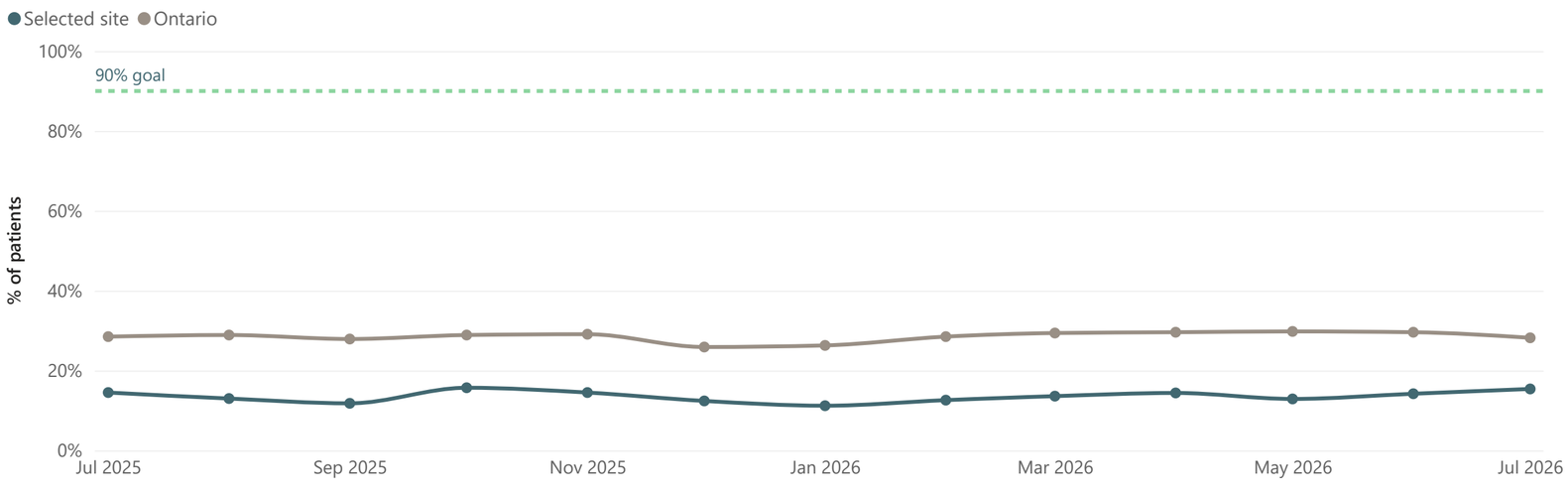
Location

Credit Valley | Mississauga | Queensway (UCC)

Average hours: Length of stay: admitted patients



Share of patients within target: Length of stay: admitted patients



How to read this page

- Use the dropdown to choose a measure; the site buttons choose the hospital.
- Dashed grey = Ontario average. Dashed green = provincial goal (90% of patients within the target time).
- *Time to first doctor assessment* has no provincial target, so the lower chart is blank for it.

What stands out

- **Mississauga** cut its admitted patients' average ED time from a peak of 32.0 h (Sep 2025) to 20.9 h (Jul 2026).
- **Credit Valley** peaked at 36.2 h in January 2026, during the winter surge, and still sits about 8 h above Ontario.
- Across Ontario, fewer than 1 in 3 admitted patients leave the ED within 8 hours in any month.

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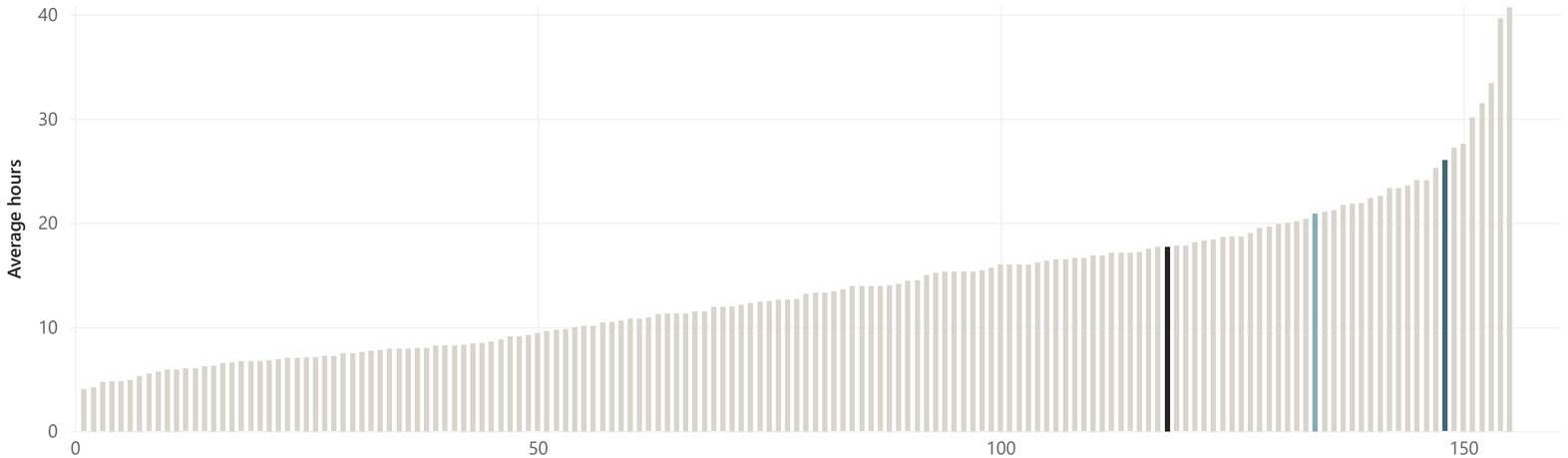
Measure

Length of stay: admitted patients

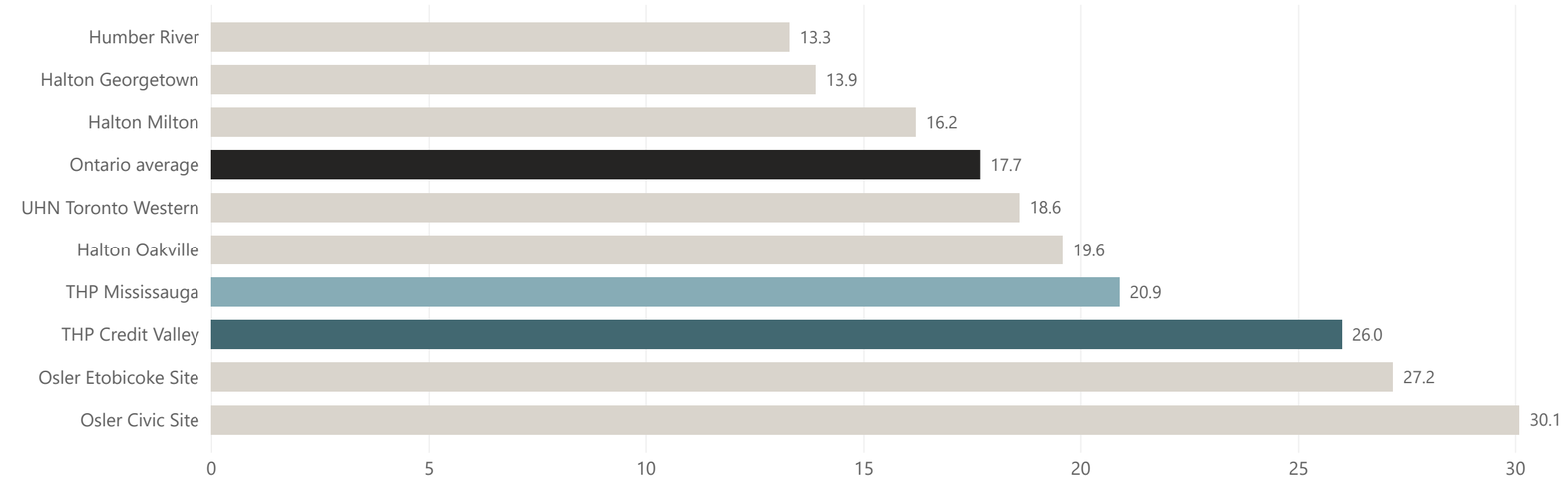
Location

Credit Valley | Mississauga | Queensway (UCC)

Every Ontario ED, shortest to longest average wait time



Trillium vs neighboring EDs (Peel, Halton, West Toronto)



Ontario rank (1 = shortest wait)

#147 of 154

Ontario EDs with a longer wait

5%

What stands out (admitted patients)

The problem is regional. Credit Valley (26.0 h) ranks #147 of 154, but both nearby Osler sites rank even lower: Etobicoke 27.2 h (#148) and Brampton Civic 30.1 h (#150). Admitted-patient delays across Peel point to regional inpatient bed pressure, not one hospital.

Neighbours do better. Humber River (13.3 h) and Halton Milton (16.2 h) beat the Ontario average of 17.7 h.

The typical Ontario ED keeps admitted patients about 12.6 h (median). Mississauga's 20.9 h is 8 hours longer.

Front-door speed is a different story. For time to first doctor, Osler Etobicoke (0.8 h) and Brampton Civic (1.2 h) rank in the provincial top 30, while Credit Valley (2.7 h) ranks #130.

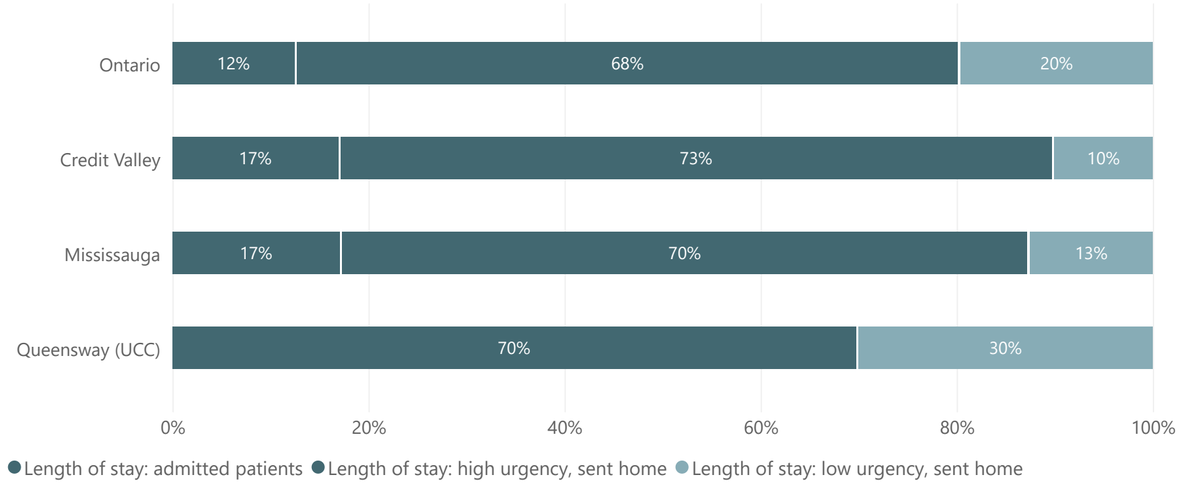
Volume & patient mix: Trillium sites vs Ontario

Latest month: July 2026 · Source: Ontario Health (NACRS, CIHI)

Measure

Time to first doctor assessment

Patient mix: share of ED visits by type (13 months)



What stands out

Same volume, same mix, different waits. Mississauga and Credit Valley each see about 9,300–9,400 ED visits a month, and about 17% of patients at each are admitted. Yet Credit Valley waits longer on every measure, so volume and patient mix don't explain the gap.

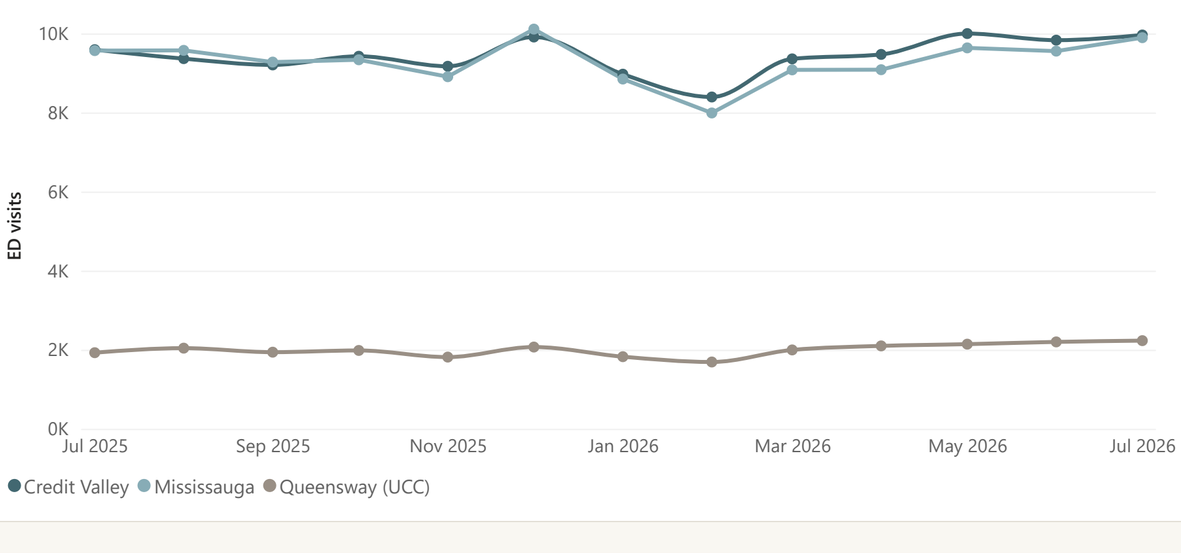
Both sites admit more patients than the Ontario average (17% vs about 12%), in line with their role as large regional hospitals.

Busier months slow the front door, not admissions. First-doctor waits rise in busier months (r ≈ 0.5–0.6), but admitted patients' time in the ED doesn't. That's consistent with inpatient bed availability, rather than ED demand, driving admitted-patient delays.

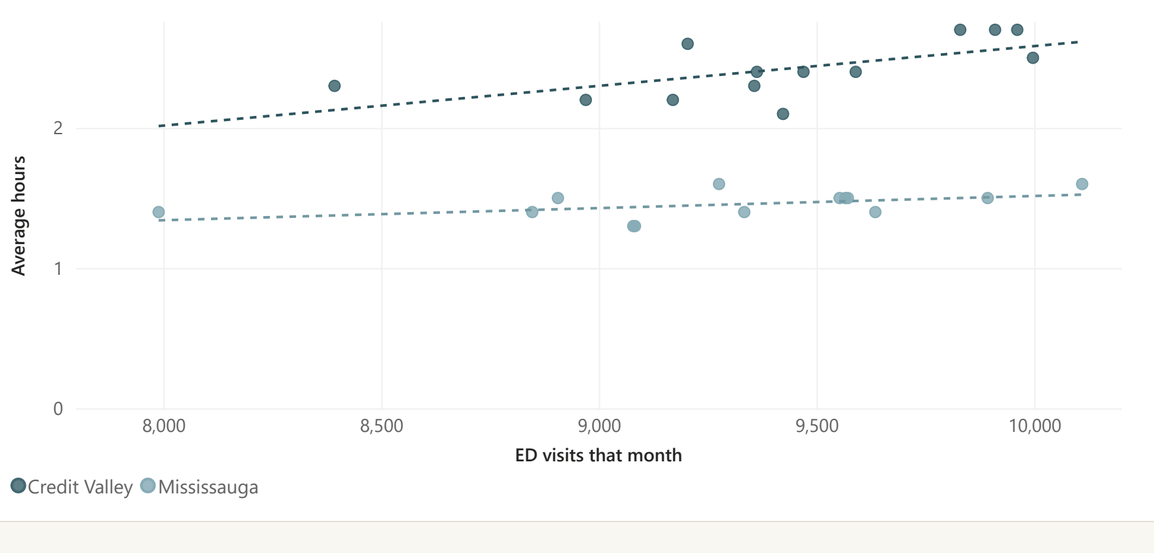
Queensway sees about 2,000 visits a month, 30% of them low urgency, and admits no one.

13 months per site: patterns are exploratory, not proof of cause.

Monthly ED visits, July 2025 to July 2026



Monthly ED visits vs average hours: Time to first doctor assessment



Methods & caveats

Data source

• **Ontario Health, "Time Spent in Emergency Departments"** public reporting, based on the National Ambulatory Care Reporting System (NACRS) from the Canadian Institute for Health Information (CIHI).
• **Trends:** monthly data from July 2025 to July 2026 (13 months) for Trillium Health Partners' three sites and the Ontario average, exported as CSV in September 2026.
• **Rankings:** the July 2026 hospital comparison table covering every Ontario ED that reports each measure (154–164 hospitals, depending on the measure).
• All data is **public and aggregated**. No patient-level information was used.

Measures and definitions

• **Time to first assessment:** average hours from arrival until a doctor or nurse practitioner first assesses the patient. There is no provincial target.
• **Length of stay (LOS):** average hours from arrival until the patient leaves the ED.
– *Low urgency, sent home* (CTAS 4–5): target 4 h
– *High urgency, sent home* (CTAS 1–3): target 8 h
– *Admitted to hospital*: target 8 h
• **% within target:** share of patients whose stay was within the target time. The provincial goal is **90%**.
• **Status:** Met = 90% or more · Near = 80–89.9% · Missed = under 80%. The "Near" band is my own analytical choice, not an Ontario Health definition.
• **Rank:** 1 = shortest average wait. Ties share a rank; Ontario isn't ranked.
• **vs last year:** July 2026 vs July 2025 (same month, to avoid seasonal effects).

How the report was built

• **Power Query:** combined 30 CSV exports into one table, split file names into site, measure and metric, converted months to dates, and reshaped the table so Ontario becomes its own row (533 rows).
• **Data model:** a measure lookup table (labels, targets, sort order) links the monthly trends and the provincial rankings.
• **DAX:** latest-month values, gap vs Ontario, target status, provincial rank, change vs 12 months.
• **Neighbouring peers:** the William Osler, Halton Healthcare, Humber River and UHN Toronto Western EDs, which serve the same Peel, Halton and west-Toronto area. The choice of peers is a judgement call.
• **Validation:** key values were cross-checked against the source files and an independent Python calculation (hidden QA page). The volume–wait correlations were also calculated in Python.

Caveats and limitations

• **Averages hide the extremes.** A few very long stays can raise an average. Ontario Health's public data doesn't include medians or 90th percentiles for each site.
• **Queensway is an urgent care centre**, not a full ED. It admits no patients (reported as "NV" for admitted measures), so it isn't directly comparable.
• **Correlation isn't causation.** The volume–wait patterns come from 13 months per site and are exploratory. The bed-availability explanation is *consistent with* the data, but no occupancy or staffing data was available to test it.
• **Patient mix is approximate.** The three patient groups are counted separately and add up to 100–102% of total visits.
• **Rankings are one month only** (July 2026) and can shift from month to month.
• **Data is reported with about a two-month lag.** Figures reflect July 2026, not current conditions.